INTERNAL TEAM BRIEFING

Sahara and CARE: where we fit

CARE is the hospital staff's system. Sahara is the family's phone app. What is different, and what we need to decide.

27 October 2026

THE ONE BIG THING

CARE is the staff side. Sahara is the family side.

CARE is a free hospital record system (an EMR) made by OHC in Kochi, Kerala. Kerala says 330,000+ patients use it [unverified] OHC's own claim

Staff side: CARE

- Doctors and nurses type in all medical data.
- Patient can log in with an OTP (phone code).
- That login can only register and book or cancel visits. It lasts 1 hour.
- Family cannot add symptoms, medicines or notes.



not connected today

Family side: Sahara

- Family adds the sick person and invites relatives.
- Symptom diary 0 to 10, 3D pain body, daily check-in.
- Medicines: list, reminders, "taken" or "missed".
- Family confirms the exact goals-of-care note.

CARE has no family side, and no door for one yet. That gap is where we fit.

Source: CARE_VS_SAHARA_FOR_TEAM.md; CARE_FAMILY_SIDE_FIT.md §1, §3.1; CARE_BRIEF §1–2. CARE code read, not run.

THE WORDS

Real words, simple meaning

CARE
The hospital's computer notebook for doctors and nurses.

EMR
Electronic Medical Record: the patient's file, on a computer. CARE is one.

OHC
Open Healthcare Network: the Kerala group that builds CARE.

Staff side / family side
Screens for doctors and nurses / screens for the family (us).

OTP
A one-time code sent to your phone, to log in.

Consent
A saved "yes" from the patient.

Verification
A second person checks that "yes".

Encounter
One visit or one hospital stay.

Questionnaire
A form with questions.

Plugin
A Lego piece that clicks into CARE.

FHIR
A shared language for health data. CARE copies its ideas; it does not speak it directly.

ABDM
India's national digital health system. Sahara is not linked to it (ABHA link: later).

Source: CARE_VS_SAHARA_FOR_TEAM.md "First, the words"; CARE_FAMILY_SIDE_FIT.md §3.2 (CARE is not a FHIR server); CHAT_AUDIT_2026-10-27.md G3 (ABHA later)

OUR SIDE OF THE DIFFERENCE

What Sahara has that CARE doesn't

SAHARA THING	CARE?	IN PLAIN WORDS
Invite family by WhatsApp	No	Send a link, relative joins. Each can edit or only read.
Medicine taken / missed	No	Family ticks each dose. CARE logs only nurse-given doses.
Medicine scan, reminders	No	Photo of prescription becomes text; phone alarm per dose.
3D pain body	Partly	12 parts, each 0 to 10. CARE: one body-part code, no picture.
Symptom trend	No	"4 now, 6 yesterday".
Goals-of-care questions	No	CARE docs say "do it separately" and "coming soon".
Record the talk	No	Each person in the room says "yes" first.
Family confirms the note	No	"I read version 3". CARE: staff mark a consent "checked with family".
Works offline	No	Recordings and reports stay on the phone. CARE is a server.

Both have: medicines list, visits, lab results, documents. Honest: our transcript is pre-written, answers use word matching, AI is off. Full: Table 1 in CARE_VS_SAHARA_FOR_TEAM.md

CARE'S SIDE OF THE DIFFERENCE

What CARE has that Sahara doesn't

CARE THING	SAHARA?	IN PLAIN WORDS
Patient details	No	Gender, birth date, phone, address, blood group. We keep only the name.
Booking, appointments	No	Slots, booking, queue tokens.
Vitals	No	Blood pressure, pulse, with standard codes.
Diagnoses, allergies	No	Coded disease list, allergies.
DNR / advance directive	No, by choice	Staff record it. We explain it and keep copies only.
Referrals, dashboards	No	Send a patient on; counts of overdue visits.
Care team, volunteers	Partly	CARE: real staff accounts. Ours: a list of names.
Hospital-built questionnaires	Partly	Hospital builds any form. Ours are fixed in the app.
Coded records, plugins	No	Medical code lists (SNOMED, LOINC). A plugin could be our way in later.

Both have: caregiver details (ours: many relatives), symptoms, medicines taken, files. CARE "care plan": on its web page, not in its code. Full: Table 2

SAME IDEA, DIFFERENT WAY

We both do it. We do it differently.

TOPIC	SAHARA	CARE	WHY WE CHOSE OURS
Pain, symptoms	0 to 10 diary: now, 24-hour average, 24-hour worst.	Mild / moderate / severe, or a hospital form.	Claim an official scale only with clinicians and permission.
Family "yes"	Family confirms the exact note version.	Staff mark a consent "checked with family".	GOCD step 4: "patient or surrogate authorizes".
Recording consent	A "yes" from each person in the room.	One consent per patient, per encounter.	PRODUCT_REQUIREMENTS.md "Source capture".
Advance directive	Explain it, keep copies, never create one.	Staff record DNR or advance directive consent.	Vidhi papers; GOCD audit "scope defect".
Log in	Email account, family invites.	OTP phone code for patients, 1 hour.	The family is our main user.
Where data lives	On the phone, works offline.	On the hospital's server.	Our decision; CARE is built for hospitals.

Kerala's own home-visit form: [unverified] · Source: CARE_VS_SAHARA_FOR_TEAM.md "Same thing, different form or scale" and "Decisions"

TALKING TO JUDGES

What not to say, and what to say

Do not say

- "Sahara integrates / syncs with CARE." Nothing connects.
- "Sahara fills CARE's consent gap." Related, not the same.
- "CARE has a family app or login."
- "FHIR-compliant export."
- "Sahara writes care plans into CARE."
- "A family yes is a verified clinical record."
- "Our diary is ESAS-r or IPOS" (official scales).

Say these 3 lines

1. CARE is the doctors' and nurses' record for palliative home care. Sahara is the family's phone app.
2. We use CARE's record types (patient, home visit, consent checked with family, forms, medicines told by family), so a hospital on CARE can take our export. Nothing is connected live yet.
3. We add what CARE leaves out: family accounts, a 3D pain diary, and the family confirming the exact goals-of-care note themselves.

Source: CARE_FAMILY_SIDE_FIT.md §6; CARE_VS_SAHARA_FOR_TEAM.md "What to say to judges in 3 lines". Line 2 assumes an export exists; today it does not (see next slide).

OPEN DECISION

CARE + Sahara: to decide in the meeting

OPTION	WHAT IT MEANS	EFFORT	RISK, IN PLAIN WORDS
A. Describe only suggested	Deck slide 11 says: Sahara is the family side and can feed a hospital's CARE.	1–2 hours	Low. Only words; we must say nothing is built.
A + B. Export file	Phone writes a CARE-shaped file a hospital can import. Not live.	1–2 days	Takes time from the demo video. CARE needs fields we don't collect.
A + C. Plugin	A small Lego piece inside CARE, e.g. a family-confirmation tab.	1–2 weeks [unverified]	High before 7 Nov. Needs a local CARE, React and Django; our Kotlin can't be reused.
D. Move onto CARE	Replace Sahara's backend with CARE.	Weeks	Very high. A rewrite that throws away a working app. Not suggested.

Questions for the team: A, A+B or A+C? Who writes the slide 11 answer? Is there time for B after the video?

What we don't know: whether judges reward building on CARE. No known judging rule mentions it. CARE is optional.

Source: CARE_BRIEF_2026-10-27.md §4–5; CARE_FAMILY_SIDE_FIT.md §5 (P1 export, P2 plugin, P3 pilot); HANDOFF.md items A17, H18. "Suggested" = the brief's view (moderate confidence), not a decision.

WHERE THIS COMES FROM

Sources

In our repo

- docs/research/CARE_VS_SAHARA_FOR_TEAM.md (full tables)
- docs/research/CARE_FAMILY_SIDE_FIT.md (field by field)
- docs/research/CARE_BRIEF_2026-10-27.md (options, organiser email)
- HANDOFF.md, "27 Oct 2026 — open items"
- CLAUDE_TRANSCRIPT.md §2
- Our app: android/app/src/main/java/in/wedevit/care/ at 7abf52e

CARE

- github.com/ohcnetwork/care at 2bae933
- github.com/ohcnetwork/care_fe at 81e7515
- ohc.network/solutions/palliative-care
- docs.ohc.network/concepts/clinical/consent/
- docs.ohc.network/flows/clinical/consent/verify-a-consent/
- docs.ohc.network/deployments/palliative/playbooks/patient-enrollment

We read CARE's code and docs (27 Oct). We did not run a CARE server. Everything marked [unverified] still needs checking.